AR-02 Adult Failed Airway

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 17, 2026

Scope: Use when the patient cannot be oxygenated or ventilated, anatomy is unsafe for continued attempts, or the most experienced Paramedic has made three unsuccessful laryngoscopy attempts.

Field flow

1. Declare	Stop repeated laryngoscopy; call for the most experienced airway provider and rescue devices.
2. Optimize BVM	Position, suction, OPA/NPA, mask seal, PEEP, high-flow oxygen, and two-person technique.
3. Rescue	If BVM remains ineffective: AEMT/Paramedic authorized BIAD; Paramedic deliberate changed video-laryngoscopy rescue or surgical airway when required.
4. Confirm	Waveform EtCO2, secure device, post-airway care, immediate transport, and early notification.

Provider-level actions

Level	Approved actions
EMT	Stop further attempts and continue BVM when it maintains SpO2 at least 92% with adequate ventilation.
AEMT	All EMT care plus authorized BIAD placement; do not perform ETT placement or video laryngoscopy. Confirm BIAD with waveform capnography.
Paramedic	Proceed to surgical cricothyrotomy only when less-invasive methods cannot oxygenate or ventilate. Avoid added laryngoscopy after three attempts or when anatomy makes success unlikely.

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SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

- If BVM maintains SpO2 at least 92% with adequate ventilation, use it as the definitive field airway and begin transport.
- Do not abandon effective BVM merely to obtain an advanced airway.
- Ventilate approximately 10-12 breaths/min with visible chest rise; use EtCO2 and clinical response to avoid hyperventilation.
- Use DOPE for sudden deterioration after airway placement.

Linked pathways

AR-01 Adult Airway; AR-03 Drug-Assisted Airway; AR-08 Post-Intubation / BIAD Management.

Source control

- This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 17, 2026.
- Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.